

APPENDICES

Appendix A: Vermont Key Data Reference Tables

This appendix consolidates the most frequently referenced Vermont healthcare data points from throughout the book into a single reference section. All data reflects the most recent available figures as of April 2026.

A.1 Vermont Demographics

Indicator	Value and source
Total population (2024 estimate)	~647,000 (US Census Bureau)
Population density	70 persons per square mile (well below national 94)
Rural population share	~65% by federal rural health definition (all locations outside Chittenden County per RHT)
Population aged 65+ (2025)	~21.7% of total population
Population aged 65+ projected (2040)	30%+ of total population — 57% increase over 2020 baseline (Oliver Wyman)
Working-age population projected change (2040)	-13% decline from current ~367,000 to ~318,000 (Oliver Wyman)
Median household income growth 2018-2024	+22% (vs. 108% premium increase in same period)
Unemployment rate (August 2025)	2.5% — well below national 4.3% (Vermont Dept. of Labor)

Source: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application (November 2025); US Census Bureau.

A.2 Vermont Hospital Financial Data (FY2023 unless noted)

Metric	Vermont PPS hospitals	Vermont CAH hospitals	National benchmark (75th %ile)	Source
Hospitals reporting operating losses	9 of 14 total (64%)	—	—	Oliver Wyman Act 167 Report
Operating profit per adjusted discharge	\$5,345	\$938	\$4,162 (PPS) / \$2,784 (CAH)	NASHP via CMS; AHS Nov 2025
Mgmt & admin cost per adjusted discharge	\$2,730	No data available	\$1,427	NASHP via CMS; AHS Nov 2025
Direct patient care labor cost per adj. discharge	\$6,183	No data available	\$6,270	NASHP via CMS; AHS Nov 2025

UVMMC admin cost per adjusted discharge	\$3,826	—	\$1,427	AHS November 2025 Report
Commercial price vs. Medicare (avg. Vermont)	250-300% of Medicare	—	~136% (break-even)	GMCB; Oliver Wyman
UVMMC outpatient price vs. Medicare	417% of Medicare (2022)	—	—	RAND Hospital Price Transparency
Outpatient imaging, highest Vermont hospital	Up to 944% of Medicare	—	—	GMCB February 2026 RBP Update
5-year cumulative deficit (Oliver Wyman conservative)	\$700M (13/14 hospitals losses by 2028)	—	—	Oliver Wyman Act 167 Report
5-year cumulative deficit (Oliver Wyman realistic)	\$2.4 billion	—	—	Oliver Wyman Act 167 Report
UVMMC 5-year deficit (realistic scenario)	\$1.5 billion	—	—	Oliver Wyman Act 167 Report

Source: Oliver Wyman Act 167 Report (August 2024); AHS Health Care System Transformation Report (November 2025); GMCB Act 68 RBP Update (February 2026).

A.3 Vermont Insurance and Premium Data

Metric	Value
Individual market monthly premium (2018)	\$456/month
Individual market monthly premium (2024)	\$948/month (+108% in 6 years)
Per-member-per-month cost (2020)	\$481
Per-member-per-month cost (2024)	\$812 (+69% in 4 years)
Out-of-pocket maximum change (5 years)	More than doubled
Vermont insured rate (2025)	97% — one of highest in country
Essex County uninsured rate	~8% vs. 3% statewide
Black adults with health insurance (VT)	82% vs. 94% statewide (VDH 2024)
FY2026 total commercial revenue reduction	\$230.65M (Act 55 drug cap \$104.3M + budget orders \$94.58M + enforcement \$31.76M)

Source: AHS January 2025 Legislative Presentation; Vermont Department of Health Health Equity Data Report (January 2025); GMCB.

A.4 Vermont Workforce Data

Metric	Value
Primary care FTE shortage projected (2030)	370 FTEs total: 112 family medicine, 190 internal medicine/other primary care, 52 OBGYNs, 2 pediatrics
Vermont rental vacancy rate	3% (healthy market = 5%)
Vermont renters who are cost-burdened	~50%
Vermont per capita homelessness rate	Second highest in the country
HRSA Health Professional Shortage Areas	None recognized in Vermont (HRSA data) — productivity/care model gap, not headcount shortage alone
Broadband access (rural Vermont)	80% of rural Vermonters (vs. 85% non-rural)

Source: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

A.5 Vermont Clinical Quality Indicators

Measure (Act 167/68 reporting)	Current VT performance
30-day all-cause readmission rate	14.8% (within national benchmark range of 14.5%)
Potentially avoidable ED visits (% of all ED visits)	32.3%
Vermonters with a personal health care provider	91% (4 points above 87% national benchmark)
Primary care practices accepting new Medicaid patients	59%
Blueprint PCMH patients with hypertension controlled	77%
Blueprint PCMH patients with diabetes HbA1c not controlled	22%
ED visits for suicide ideation/self-harm per 10,000 ED visits	245.5 (higher in Rutland, Windham counties; ages 15-44)
ED visits for opioid overdose per 10,000 ED visits	16.1 (higher in Chittenden, Bennington, Windham counties)
30-day follow-up after MH ED visit	76%
30-day follow-up after SUD ED visit	68%

Source: AHS Health Care System Transformation Report (November 2025); Blueprint for Health 2024 data.

Appendix B: Act 68 Statutory Timeline and Milestones

Act 68 of 2025 (S.126) established a comprehensive sequence of healthcare transformation milestones. This appendix consolidates the complete timeline for reference.

Date / Deadline	Statutory requirement	Primary authority	Status (April 2026)
January 1, 2026	AHEAD Medicaid global budgets begin (Cohort 2 cooperative agreement preparation; Medicaid alignment)	DVHA / CMS	Operational — Medicaid HGB in effect
FY2026 (ongoing)	Hospital budget orders implementing -1% commercial rate benchmark	GMCB	In effect — \$94.58M in FY2026 budget reductions ordered
FY2026 (ongoing)	\$2M Act 68 transformation grants to hospitals (HIT fund)	AHS	Launched September 2025 — all eligible hospitals submitted applications
FY2026 (ongoing)	Act 55 drug pricing cap (\$135M savings target)	GMCB / DFR	In effect — \$104.3M in FY2026 drug cap savings
Early 2026	Final hospital Care Transformation Plans approved by AHS	AHS	In progress — hospitals in draft plan phase
May 2026	BALANCE Model GLP-1 Medicaid launch (voluntary state participation)	DVHA / CMS	DVHA participation decision pending
July 1, 2026	Five new CCBHC entities certified (RHT Program target)	AHS / DMH	On track per RHT application schedule
July 2026	Medicare GLP-1 Bridge demonstration launches (\$50/month co-pay)	CMS	Federal program — Vermont Medicare beneficiaries eligible
FY2027 (October 1, 2026)	Reference-based pricing (RBP) mandatory for commercial payers — maximum rates set by GMCB	GMCB	GMCB developing RBP methodology; vendor procurement underway
January 1, 2027	AHEAD Model Cohort 2 begins — Medicare FFS global budgets for Vermont	AHS / DVHA / GMCB / CMS	9-year model; primary care AHEAD and hospital global budgets launch
January 2027	BALANCE Model Medicare Part D coverage launches	CMS	Federal program
FY2028 (October 1, 2027)	Global hospital budgets for non-CAH (PPS) Vermont hospitals	GMCB	Methodology under development; AHEAD alignment required
December 1, 2028	Statewide Health Care Delivery Strategic Plan delivered to Vermont Legislature	AHS + Health Care Delivery Advisory Committee	In planning — plan development begins 2026
FY2030 (October 1, 2029)	Global hospital budgets for ALL Vermont hospitals including CAHs	GMCB	Final phase of global budget implementation
December 2031	BALANCE Model testing concludes	CMS	Federal program

October 1, 2032	All RHT Program funds must be spent	AHS / CMS	\$195M award; spending window 2026-2032
Every 3 years from 2028	Statewide Strategic Plan updated	AHS + Advisory Committee	Ongoing statutory obligation

Source: Act 68 of 2025 (Vermont); AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); CMS BALANCE Model (December 2025).

Appendix C: The 14-Hospital Centers of Excellence Assignment Framework

Oliver Wyman's Act 167 Recommendations proposed a Centers of Excellence (COE) framework organizing Vermont's 14 hospitals by specialty service concentration. This appendix presents the proposed assignments.

Hospital (HSA)	COE count	Key COE specialties assigned	Tier designation	RHT rural status
University of Vermont Medical Center (Burlington)	16	All major specialties including cardiac surgery, neurosurgery, complex oncology, neonatal intensive care, transplant, trauma	Tier 1 — Primary RSC	Non-rural (Chittenden County)
Southwestern Vermont Medical Center (Bennington)	9	Acute general surgery, cardiology, orthopedics, psychiatry (adult and adolescent), rehabilitation, dermatology, endocrinology, GI, pain management	Tier 1 — Secondary RSC	Rural
Brattleboro Memorial Hospital (Brattleboro)	6	Acute general surgery, cardiology, obstetrics/high-risk OB, psychiatry (adult), oncology outpatient, GI	Tier 1 — Secondary RSC	Rural
Rutland Regional Medical Center (Rutland)	5	Neurology, psychiatry (adult), cancer surgery, radiation therapy, orthopedics	Tier 1 — Secondary RSC	Rural
Central Vermont Medical Center (Barre)	5	Radiation therapy, neurology, cardiac, general surgery, obstetrics	Tier 1 — Secondary RSC	Rural
Northwestern Medical Center (St. Albans)	4	Cancer surgery, radiation oncology, orthopedics, cardiology	Tier 2 — Focused scope	Rural
Northeastern Vermont Regional Hospital (St. Johnsbury)	4	Cancer surgery, infusion/chemotherapy, orthopedics, general surgery	Tier 2 — Focused scope	Rural
Springfield Hospital (Springfield)	2	Memory care/dementia, psychiatry (adult)	Tier 2 — Focused scope	Rural
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology	Tier 2 — Focused scope	Rural
Mount Ascutney Hospital (White River Junction)	1	Rehabilitation	Tier 2 — Focused scope	Rural
Porter Medical Center (Middlebury)	TBD	Assessment ongoing — UVM network affiliation	Tier 2 — Focused scope	Rural
Grace Cottage Hospital (Townshend)	TBD	Palliative care specialty worth preserving; 19-bed Vermont's smallest hospital	Tier 3 — Transition assessment	Rural
Gifford Medical Center (Randolph)	TBD	Most financially vulnerable — comprehensive assessment needed	Tier 3 — Transition assessment	Rural

North Country Hospital (Newport)	TBD	Northeast Kingdom anchor; REH candidacy under assessment	Tier 3 — Transition assessment	Rural
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Source: Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025). Note: COE designations are proposed frameworks from Oliver Wyman analysis; final designations subject to AHS and hospital transformation planning process.

Appendix D: AHEAD State Agreement — Key Terms and Provisions

Vermont's AHEAD Model State Agreement was signed January 17, 2025. Vermont participates as a Cohort 2 state. This appendix summarizes the agreement's key terms for practitioner reference.

AHEAD provision	Vermont-specific terms
Cohort and start date	Cohort 2; model begins January 1, 2027; 9-year model through December 31, 2035
Total Cost of Care accountability	Vermont responsible for all-payer TCOC targets; Medicare FFS TCOC tracked annually; consequences for failure to meet targets after year 3
Hospital global budgets (Medicare FFS)	All Vermont hospitals receive prospective Medicare FFS global budgets beginning January 2027; methodology developed jointly with GMCB
Primary Care AHEAD (PC AHEAD)	Voluntary program for Vermont primary care practices; four payment pathway options including fully prospective models; replaces Blueprint Medicare PMPM payments
EAST Fund	Up to \$150M per year in additional Medicare funds for Vermont to invest in primary care, behavioral health, home health, long-term care, and SDOH — the transformation capital fund
Behavioral health and substance use	EAST Fund explicitly directed toward MH and SUD services; CMMI-approved \$1.2M in transformation grants from Cooperative Agreement Year 3 (2027)
Vermont GMCB regulatory independence	Agreement explicitly preserves GMCB's authority to set hospital rates; AHEAD does not supersede Vermont's state regulatory authority
Withdrawal rights	Vermont may withdraw before launch with 30-day notice; after launch with 6-month notice; unspent cooperative agreement funds must be returned to CMMI within 90 days of withdrawal
Cooperative agreement funds (pre-launch)	CMS provides cooperative agreement funds for pre-launch implementation activities; \$1.2M approved for Cooperative Agreement Year 3 (2027)
Quality performance	Vermont must meet quality performance standards; quality measures aligned with Blueprint, HEDIS, and Act 167/68 reporting requirements
CMMI evaluation	CMS conducts independent evaluation of Vermont's AHEAD performance; results inform national AHEAD expansion decisions
Relationship to Act 68	AHEAD provides federal vehicle for implementing Vermont's statutory global budget mandate; Act 68 provides Vermont's independent statutory authority that does not depend on AHEAD continuation

Source: Vermont AHEAD State Agreement (January 17, 2025); CMMI AHEAD Model documentation; AHS January 2025 Legislative Presentation.

Appendix E: HTR Platform Tools and Methodology Overview

This appendix describes the Health Transformation Review platform tools referenced throughout the book. All tools are available to Research Lab subscribers at htreview.com.

E.1 APM Shared Savings Calculator

Models shared savings or loss potential under any APM contract. Inputs: benchmark PMPM, attributed member months, actual total cost of care, sharing rate, minimum savings rate, quality withhold percentage, and stop-loss threshold. Outputs: projected shared savings or loss under pessimistic/base/optimistic scenarios, sensitivity analysis by key variable, and break-even care management investment level.

Methodology: Standard CMS shared savings calculation framework adapted for multiple model types (MSSP, ACO REACH, AHEAD global budget, state APMs). Vermont-specific data pre-loaded for 14 hospitals and Blueprint practices.

E.2 VBC Transformation Readiness Assessment

A 30-dimension, 6-domain assessment scoring organizational readiness for APM contract entry on a 1-4 scale per dimension (total 120 points). Domains: Strategic Clarity, Data and Technology, Care Delivery Capability, Network and Partnerships, Revenue Cycle, and Workforce Operations. Available as self-assessment (Research Lab) or facilitated external assessment (Advisory).

Scoring: Below 60 = Pre-transition (not ready for full-risk); 60-80 = Developing (selective VBC readiness); 80-100 = Operational (ready for most VBC models); 100-120 = Optimized (ready for full-risk capitation).

E.3 Health Equity Studio (HEROI)

Calculates the Health Equity Return on Investment (HEROI) composite score across five dimensions: Access Equity (25%), Quality Equity (25%), Outcome Equity (25%), SDOH Burden (15%), and Trust and Engagement (10%). Integrates with VHCURES exports and Blueprint data for Vermont providers. Includes disparity decomposition module that separates observed disparities into root cause categories (geographic access, social needs, clinical variation, coverage barriers, trust barriers) for targeted intervention design.

E.4 Hospital Financial Stress Test Model

Models hospital financial position under RBP, global budget, and Medicaid cut scenarios with organization-specific inputs. Key inputs: current net patient revenue by payer, current commercial price vs. Medicare ratio, operating cost structure, payer mix, and projected Medicaid revenue changes. Outputs: 10-year financial projections under three scenarios (status quo trajectory, partial transformation, full transformation) with sensitivity analysis.

E.5 AI Clinical Governance Checklist

65-item governance evaluation across six lifecycle stages: Pre-Deployment Evaluation, Implementation and Training, Active Monitoring, Adverse Event Response, Vendor Management, and Equity Monitoring. Includes FDA SaMD classification guidance, bias

testing requirements by data type, override rate thresholds, and vendor contract template provisions.

E.6 VBC Contract Review Checklist

65-item analysis framework organized in eight categories: Benchmark Methodology, Attribution, Quality Withhold, Risk Corridors and Stop-Loss, Carve-Outs, Reconciliation, Reporting Requirements, and Termination Provisions. Identifies the specific contract terms that most commonly produce unexpected financial outcomes for organizations entering VBC arrangements.

E.7 Policy Impact Assessment Framework

Structured evaluation tool for new policy developments: (1) Exposure mapping — what does this policy require of the organization? (2) Financial impact estimation — what are the revenue, cost, and margin implications? (3) Scenario analysis — how do outcomes change under different implementation assumptions? (4) Response strategy selection — compliance, comment letter, advocacy, CMMI application, or strategic repositioning? (5) Stakeholder communication — who needs to know, what decisions does it require, and what is the communication timeline?

E.8 FHIR Implementation Guide

Step-by-step technical guide for implementing Vermont's three priority FHIR use cases: Patient Access API (§170.315(g)(10) compliance), Provider Access API for care coordination, and Payer-to-Provider API for prior authorization. Includes EHR-specific configuration notes for Epic, Oracle Health/Cerner, Meditech Expanse, and TruBridge. Covers master patient index design, patient matching methodology, and CommonWell/Carequality network connection requirements.

E.9 HCC Gap Closure Playbook

Methodology guide for retrospective HCC gap analysis. Covers: RAF comparison methodology (organization vs. regional benchmark), chart review protocol for identifying undocumented chronic conditions, HCC specificity coding education modules for 20 highest-volume HCC categories, AWV completion program design, and monitoring dashboard for ongoing gap closure tracking. Vermont-specific: pre-loaded AHEAD Medicare attribution data and VHCURES-derived RAF benchmarks for Vermont peer comparison.

E.10 HTR Intelligence Feed

Weekly synthesis of consequential healthcare transformation developments, published every Tuesday. Coverage organized around the six pillars: Policy (federal rulemaking, CMMI model developments, state legislation), Economics (VBC market developments, hospital financial trends, payment model results), Technology (health IT developments, AI governance, interoperability standards), Clinical (quality measurement, care model evidence), Equity (disparity data, equity policy), and Operations (workforce, administrative simplification). Free tier available at htreview.com; Professional and Advisory tiers include full analysis, Vermont-specific commentary, and database access.

For subscription information and tool access, visit htreview.com or email info@htreview.com.